

PATIENT INFORMATION

Patient Name: Mr. Rajesh Kumar Sharma Patient ID: APL-PT-20240083
Date of Birth: 14-March-1978 (Age: 48 yrs) Gender: Male
Blood Group: B+ Contact: +91-98765-43210
Address: 42, Indiranagar 1st Stage, Bengaluru – 560038, Karnataka
Referred By: Dr. Priya Menon, MD Department: Internal Medicine
Sample Collected: 22-May-2026, 07:30 AM Report Generated: 22-May-2026, 11:45 AM

CLINICAL SUMMARY

The patient presented with complaints of fatigue, mild breathlessness on exertion, and occasional dizziness for the past 3 weeks. He has a known history of Type 2 Diabetes Mellitus (diagnosed 2019) and hypertension (diagnosed 2021), currently managed with Metformin 500mg BD and Amlodipine 5mg OD. No known drug allergies. Non-smoker. Occasional alcohol use.

VITAL SIGNS

Blood Pressure	Heart Rate	SpO2	BMI
138/88 mmHg	88 bpm	97%	27.4 kg/m ²

Weight: 78 kg Height: 169 cm Temperature: 98.6°F Respiratory Rate: 16/min

LABORATORY INVESTIGATIONS

Complete Blood Count (CBC)

Test	Result	Unit	Reference Range
Haemoglobin (Hb)	10.2 ↓ LOW	g/dL	13.5 – 17.5
RBC Count	3.8 ↓ LOW	mill/μL	4.5 – 5.5
WBC (Total Leucocyte Count)	7,400	cells/μL	4,000 – 11,000
Platelets	2,10,000	/μL	1,50,000 – 4,00,000
MCV	72.4 ↓ LOW	fL	80 – 100
MCH	24.1 ↓ LOW	pg	27 – 32
MCHC	30.8 ↓ LOW	g/dL	32 – 36
Haematocrit (PCV)	35.2 ↓ LOW	%	40 – 52
Neutrophils	62	%	50 – 70
Lymphocytes	28	%	20 – 40
Eosinophils	4	%	1 – 6
Monocytes	6	%	2 – 8

Biochemistry Panel

Test	Result	Unit	Reference Range
Fasting Blood Glucose	142 ↑ HIGH	mg/dL	70 – 100
HbA1c (Glycated Haemoglobin)	7.8 ↑ HIGH	%	< 5.7 (Normal)
Serum Creatinine	1.1	mg/dL	0.7 – 1.2
Blood Urea Nitrogen (BUN)	18	mg/dL	7 – 20
eGFR	72	mL/min/1.73m ²	> 60
Serum Sodium (Na+)	138	mEq/L	136 – 145
Serum Potassium (K+)	4.2	mEq/L	3.5 – 5.1
Total Cholesterol	218 ↑ HIGH	mg/dL	< 200
LDL Cholesterol	138 ↑ HIGH	mg/dL	< 100
HDL Cholesterol	38 ↓ LOW	mg/dL	> 40 (Male)
Triglycerides	182 ↑ HIGH	mg/dL	< 150
Serum Ferritin	8.2 ↓ LOW	ng/mL	12 – 300
Serum Iron	44 ↓ LOW	µg/dL	60 – 170
TIBC	420 ↑ HIGH	µg/dL	250 – 370

CLINICAL IMPRESSION & DIAGNOSIS

Based on clinical examination and laboratory findings:

1. Iron Deficiency Anaemia (IDA)

Low Hb (10.2 g/dL), microcytic hypochromic indices (low MCV, MCH, MCHC), reduced serum ferritin (8.2 ng/mL) and serum iron with elevated TIBC — consistent with iron deficiency.

2. Type 2 Diabetes Mellitus — Suboptimal Control

FBG 142 mg/dL and HbA1c 7.8% indicate inadequate glycaemic control despite current regimen. Dose optimisation advised.

3. Dyslipidaemia

Elevated total cholesterol, LDL, and triglycerides with low HDL. Cardiovascular risk stratification recommended.

4. Hypertension — Borderline Control

BP 138/88 mmHg; continue current antihypertensive therapy with lifestyle modification.

TREATMENT RECOMMENDATIONS

Medications:

- Ferrous Sulphate 200mg OD (with Vitamin C) for 3 months; recheck Hb and ferritin after 6 weeks.
- Increase Metformin to 1000mg BD; consider adding DPP-4 inhibitor — consult endocrinologist.
- Initiate Atorvastatin 20mg OD for LDL reduction; review in 6 weeks.
- Continue Amlodipine 5mg OD; review BP at next visit.

Diet & Lifestyle:

- Increase dietary iron: green leafy vegetables, legumes, lean red meat, fortified cereals.
- Diabetic diet: limit refined carbohydrates, adopt low glycaemic index foods.
- Heart-healthy diet: reduce saturated fats, increase omega-3 fatty acids and fibre.
- Moderate aerobic exercise (30 min, 5 days/week); avoid vigorous exercise until anaemia improves.
- Weight reduction target: BMI < 25 kg/m².

Follow-up:

- Review in 6 weeks with repeat CBC, FBG, HbA1c, and lipid profile.
- Refer to Endocrinology for optimisation of diabetic management.
- Cardiology referral if lipid targets not met at 6-week review.

PHYSICIAN SIGN-OFF

Dr. Priya Menon, MD (Internal Medicine)

MBBS, MD – General Medicine

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Report Verified By:

Dr. Arvind Shenoy, MD (Pathology)

Senior Consultant – Clinical Pathology

Reg. No: KMC-2005-08831

This report has been digitally verified and is authenticated for clinical use.